

Who built the 16-bed limit

Tracing the Medicaid institution for mental diseases exclusion to the people, the votes, and the price tag that keep it alive. Every statutory quotation below was transcribed from the Statutes at Large, not from a summary.

SIX FINDINGS

- 1 Medicaid did not create this rule — it inherited it.** Mental institutions were first named in federal law in 1950, fifteen years before Medicaid existed. In 1965 Congress actually *narrowed* it.
- 2 The Senate voted to abolish it in 1960. A conference committee killed it overnight.** That is the single moment this rule was ever genuinely contested — and it was decided by a handful of conferees, not a floor vote.
- 3 The number 16 was enacted as a “technical correction”** in 1988, under a section heading that says exactly that — and it had *no predecessor* in the regulation it claimed to clarify.
- 4 Congress excluded tuberculosis hospitals in the same sentence — and deleted that half in 1984.** It has never deleted the psychiatric half.
- 5 In 2023 the House voted 386–37 to lift part of it. The Senate never took it up.** No senator voted against it. It died on a committee calendar.
- 6 CBO says full repeal costs \$38.4 billion.** Every version Congress has ever passed sits on the cheap side of that number. That, more than any person or party, is the answer.

FIRST, PRECISELY WHAT THE RULE DOES

Medicaid does fund mental health care — outpatient treatment, and psychiatric units inside general hospitals. What the exclusion bars is federal Medicaid payment for an adult aged 21–64 who is a patient in a facility *primarily* engaged in treating mental disease with more than 16 beds. Narrower than “Medicaid won’t pay for psychiatric care” — and devastating exactly where inpatient beds live, because almost no psychiatric hospital has 16 beds or fewer.

The real lineage

The chain runs thirty years before Medicaid. Each step was verified against the enacted statute.

1935 Social Security Act — and it is not about mental illness

Title I bars aid to anyone who *"is not an inmate of a public institution."* That is a poorhouse rule aimed at almshouses. The words `insane` and `asylum` appear zero times in the House Ways & Means report or the Committee on Economic Security's report to the President. Aid to Dependent Children carried no such clause — the exclusion was an adult-category rule from birth.

1950 The actual origin point

P.L. 81-734 §303(a), 64 Stat. 549, creates federal "vendor payments" to medical providers — then carves two categories back out. Mental institutions enter federal law here, as an exception to a liberalization:

...but does not include any such payments to or care in behalf of any individual who is an inmate of a public institution (except as a patient in a medical institution) or any individual (a) who is a patient in an institution for tuberculosis or mental diseases...

House conference report vote: 373–1. Republicans voted 137–1 in favour. Senate 82–2. Signed by Truman.

1960 The one time it was nearly killed

From the Social Security Administration's own legislative chronology:

August 23, 1960 — "An amendment introduced by Senator Long to permit Federal matching of vendor payments... to public mental and tuberculosis hospitals was **adopted**."

August 24, 1960 — "The Conference Committee agreed to the medical care provisions added by the Senate, except for Senator Long's amendment."

The United States Senate voted to end this rule in 1960, and a conference committee reversed it in a single day. Independently corroborated by reading the enacted statute: no provision authorising such matching survives in P.L. 86-778. Only a 42-day allowance for general-hospital care remained. If you want one identifiable decision point, this is it.

1965 Medicaid inherits it — and narrows it

P.L. 89-97 §121(a), 79 Stat. 351–352, carries the 1950 sentence into the new Title XIX, now bounded by age, while §1905(a)(14) affirmatively covers people 65 and over in an IMD. SSA's own contemporaneous account calls this *“remov[ing] the exclusion... with respect to payments for aged individuals.”*

“(B) any such payments with respect to care or services for any individual who has not attained 65 years of age and who is a patient in an institution for tuberculosis or mental diseases.”

Sixty-one years later those words are unchanged, except that “tuberculosis or” is gone. In 1965 there was no under-21 carve-out either — the exclusion hit *everyone* under 65. The 21–64 band was created in 1972.

The names on the record

These are the people the documents actually name. Where no individual is identifiable, that is stated rather than filled in.

Person	Role	What the record shows
Rep. Wilbur D. Mills Democrat — Arkansas	Sponsor of H.R. 6675; Chairman, House Ways & Means	Wrote the bill that carried the exclusion into Medicaid. Also the “Mills” of Kerr-Mills in 1960.
Sen. Harry F. Byrd, Sr. Democrat — Virginia	Chairman, Senate Finance	Reported the bill 12–5 while <i>joining four Republicans against it</i> . Opposed Medicare, not the exclusion.
Sen. Russell Long Democrat — Louisiana	Author of the 1960 amendment	The only member on record who tried to end this rule. His amendment passed the Senate and was killed in conference the next day.
Rep. Fortney “Pete” Stark Democrat — California	Sponsor of H.R. 2470 (1988)	Sponsored the vehicle that carried “more than 16 beds.” No evidence he authored that paragraph — it sits in a technical-corrections title.
Rep. Brett Guthrie Republican — Kentucky	Sponsor of H.R. 4531 (2023)	Carried the 386–37 House bill to lift the exclusion for addiction treatment. Died in the Senate.
The 1988 drafter	Author of the 16-bed paragraph	Not identifiable in the record
The 1960 conferees	Killed the Long amendment	Not individually recorded

The votes

Reconstructed from the Voteview congressional-votes database and SSA's vote tallies. The House Clerk publishes no roll calls before 1990, so these are the authoritative machine-readable record. Party splits are computed from individual member cast codes.

Law	Vote	Tally	Dem	Rep
1950 — origin of the carve-out	House conf. report	373–1	233–0	137–1
1960 — Kerr-Mills	Senate passage	74–11	43–10	31–1
1965 — Medicaid	House passage	313–115	248–42	65–73
1965 — Medicaid	Senate passage	68–21	55–7	13–14
1972 — psych under 21	House conf. report	305–1	176–0	129–1
1984 — TB deleted	House conf. report	268–155	192–69	76–86
1988 — “16 beds” enacted	House conf. report	328–72	230–9	98–63
1988 — “16 beds” enacted	Senate conf. report	86–11	52–0	34–11

Why none of these is “a vote for the 16-bed limit”

Every row is a vote on an omnibus. A member voting yes in April 1965 was voting to create Medicare. A member voting no was voting against Medicare, not for the exclusion — the Republican substitute would not have removed it either. In sixty-one years the exclusion has never once been put to a standalone recorded vote. That is the finding, not a hole in it.

How that negative was measured

“There was no vote” is a claim about a complete set, so the set was enumerated. All 394 House and 497 Senate roll calls of the 89th Congress were pulled and filtered to H.R. 6675: exactly 16 roll calls, and every description read. The House took only three recorded votes on the bill in its entire life — recommit, passage, conference report — and none on any amendment at all.

Then the amendments themselves: all 513 Senate amendments were read from the official conference print. Every Title XIX provision touching institutions for mental diseases —

amendments 255, 262–263, 275, 276, 279 — is labelled a **Committee** amendment. Not one reached the floor. The only floor amendment anyone offered on a psychiatric exclusion was Senator McCarthy's, it concerned *Medicare*, and it drew no roll call either.

Positive control: the same method surfaced 13 Senate roll calls on other specific 1965 provisions — including a 26–64 vote to strike Medicare Parts A and B outright — and did flag McCarthy's psychiatric-exclusion amendment. An instrument that found those found zero for the Medicaid IMD exclusion. **Document gate:** that 32-page conference print is a pure image scan with no text layer; a keyword search would have returned zero and meant nothing, so it was read page by page.

Where the number 16 came from

1976

A group-home rule

SSI law exempts *"a publicly operated community residence which serves no more than 16 residents."*
The earliest 16 in this corner of federal law.

1978

The IMD rule has no number

43 FR 45204 defines an IMD by *"overall character"* — no bed count anywhere. The 16 appears in that same rule only for community residences.

1988

16 becomes a wall

OBRA '87 had just emptied §1905(i). A 1988 "clarification" fills it with *"more than 16 beds."*

1991

The regulation catches up

56 FR 8854 edits the rule to match: *"'an institution' is changed to 'a hospital, nursing facility, or other institution of more than 16 beds'."* Two years and eight months *after* the statute.

A HYPOTHESIS THIS RESEARCH KILLED

The intuitive story — that career regulators invented 16 and Congress later rubber-stamped it — is wrong, and the paper trail runs the other way. Congress wrote the number in July 1988; HCFA copied it into the Code of Federal Regulations in March 1991, in a rule expressly titled *"OBRA '87 Conforming Amendments."* The agency was conforming to Congress, not the reverse. So the "technical correction" that carried the number was not correcting anything — there was nothing yet to correct.

WHERE IT SITS IN THE 1988 LAW

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SEC. 411. TECHNICAL CORRECTIONS TO CERTAIN HEALTH CARE PROVISIONS
(k) CORRECTIONS TO SUBTITLE B OF TITLE IV (RELATING TO MEDICAID)
    (13) TREATMENT OF EDUCATIONALLY-RELATED SERVICES
    → (14) CLARIFICATION OF TERM "INSTITUTION FOR MENTAL DISEASES"
    (15) ELIGIBILITY VERIFICATION TECHNICAL CORRECTION
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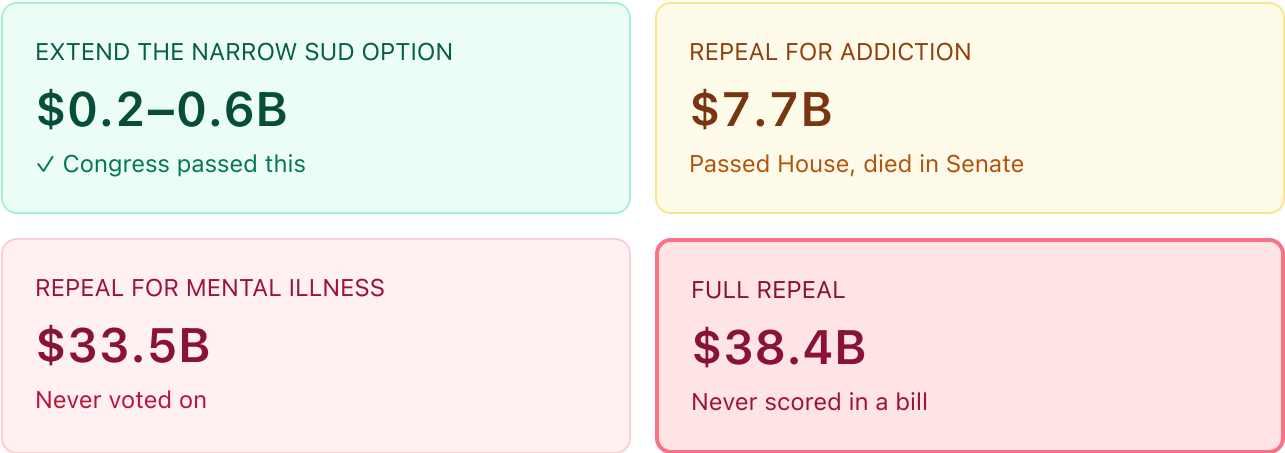

Sandwiched between a schools provision and a paperwork fix. The 1976→1988 link is a documented structural parallel, **not** a documented causal chain — no paper trail says Congress borrowed the SSI figure. What is established: the number did not come from the IMD regulation, and no clinical or empirical basis for it appears anywhere in the record searched.

Why it is still here

This is the part that actually answers the question.

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Net increase in federal Medicaid spending, 2024–2033:



Every IMD change that has ever become law sits on the cheap side of that line. Every repeal bill sits on the expensive side, and not one has ever been reported out of committee, scored, or given a vote in any Congress. The gap between what passes and what doesn't is roughly 70x to 250x.

2023: the closest it has ever come

H.R. 4531 (118th Congress) carried a section titled, verbatim, **LIFTING THE IMD EXCLUSION FOR SUBSTANCE USE DISORDER**. It cleared subcommittee 49–0, full committee 29–3, and passed the House 386–37 on 12 December 2023 — Republicans 190–26, Democrats 196–11. It went to the Senate HELP Committee the next day and never moved again. No hearing, no markup, no vote, no recorded opponent.

The substance was rescued three months later inside an appropriations bill, which made the narrow 30-day addiction option permanent. The broader relief was not.

What is live right now

Bill (119th Congress)	Sponsor	Cosponsors	Status
H.R. 5462 — Michelle Alyssa Go Act	Rep. Dan Goldman (D–NY–	14 (10 D / 4 R)	Referred to E&C. No subcommittee referral, no

Bill (119th Congress)	Sponsor	Cosponsors	Status
	10)		hearing.
H.R. 6727 — Repealing the IMD Exclusion Act	Rep. Ritchie Torres (D–NY-15)	0	Referred to E&C.
H.R. 4022 — Increasing Behavioral Health Treatment Act	Rep. Salud Carbajal (D–CA-24)	7 (5 D / 2 R)	Referred to E&C.

The Michelle Alyssa Go Act has been introduced in three consecutive Congresses and has **never** had a Senate companion. Its cosponsor count and its bipartisanship have both gone *down* since the 118th. No CBO score exists for any of these — a bill has to leave committee to get one.

So which party did this?

The voting record does not support a partisan answer. Every enactment carrying this exclusion passed with majority Republican support, under Democratic and Republican presidents alike.

- **1950**, when mental institutions were first carved out: House Republicans voted 137–1 in favour.
- **1960**, the one contested moment: the amendment to abolish it was killed by a conference committee, not a party. More Democrats than Republicans opposed the underlying bill.
- **1984**: a Democratic House and a Republican Senate together deleted the tuberculosis half and kept the psychiatric half. Nobody in either party moved to delete both.
- **1988**: the number 16 rode in on a conference report backed by 98 Republicans and 230 Democrats, signed by Ronald Reagan.
- **2023**: the House voted to lift part of it 386–37 — overwhelming majorities of *both* parties. It still died.

If you are looking for someone who destroyed this, the record refuses to give you one. What it gives you is worse and more useful: a 1950 cost-allocation decision between the states and Washington that nobody has ever had to defend on the floor, because nobody has ever been made to vote on it — protected now by a \$38.4 billion price tag and a constituency too sick and too poor to hire lobbyists. It survives on procedure and arithmetic, not ideology. That is why it has outlasted every majority of the last sixty-one years.

What the record does not contain

Measured absences, not assumptions. Each was searched with a method proven to find comparable material first.

Searched	For	Result	Control
1935 W&M report + CES report to the President	Any mention of insanity or asylums	Zero	All 10 "institution" hits found
1935, 1950, 1965 statutes + contemporaneous SSA accounts	Any stated reason for the exclusion	None at any step	Rule itself located in each
43 FR 45204, first 400 pp	"16 beds" in the 1978 IMD definition	Zero	Page 45204 confirmed in extract
Every CFR amendment to the IMD definition, 1978–1996	When the regulation acquired the number	1991, after the statute	Amendment chain read in full
Pre-1978 regulation (45 CFR 248.60)	Whether any earlier bed count existed	Not searched No CFR before 1996 online	—
All 394 House + 497 Senate roll calls, 89th Congress; all 513 Senate amendments	Any vote on the IMD exclusion	Zero	13 other amendment roll calls found
MACPAC Report to Congress on IMDs (128 pp)	Any rationale for "16"	2 mentions, both descriptive	"beds" → 15 hits
2025 SUPPORT Act reauthorisation, all 6 text versions	Any IMD or Medicaid provision	Zero	"opioid" → 36 hits
CRS report IF10222 · congress.gov	Congressional intent for the threshold	Unmeasured	Bot wall; not bypassed

One correction worth stating plainly. The familiar story — that Congress excluded asylums because President Pierce established in 1854 that they are a state responsibility — is a reconstruction by later scholars. Pierce's veto is real and says what it is quoted as saying. But no 1935, 1950, or 1965 primary

document reached here connects the two, and the 1935 record actively cuts against it: those drafters were legislating about almshouses and never mentioned insanity at all.

Sources

All fetched 1 September 2026.

Document	Used for	Status
P.L. 74-271, 49 Stat. 620	The 1935 “inmate of a public institution” rule	Confirmed
P.L. 81-734, 64 Stat. 549	The 1950 origin of the mental-institution carve-out	Confirmed
SSA legislative chronology, 1960	The Long amendment and its death in conference	Confirmed
P.L. 89-97, 79 Stat. 351–352	The exclusion as Medicaid received it	Confirmed
P.L. 100-360, 102 Stat. 798–799	§411(k)(14)(A) — the 16-bed “clarification”	Confirmed
43 FR 45204	Proof the 1978 IMD rule had no bed count	Confirmed
42 U.S.C. §1396d · §1382	Current text; amendment ledger; the 1976 SSI 16	Confirmed
CBO Publication 59071 (April 2023)	The repeal price tags	Confirmed
MACPAC Report to Congress on IMDs (2019)	The stated rationale; policy chronology	Confirmed
Voteview roll-call database · SSA vote tallies	All tallies and party splits	Confirmed
congress.gov HTML and API	Bill status (substituted GovInfo BILLSTATUS)	Blocked

Caveats, stated so you can discount them yourself. **Vote sourcing:** GovTrack draws its pre-1990 roll calls from Voteview, so they are one lineage, not two. The 1965 *totals* and the two *conference-report* party splits are independently confirmed by SSA’s official tally page; the 1965 passage-vote party splits rest on the Voteview lineage alone. Voteview’s member-level file and its own summary field also disagree by one vote on several 1965 and 1972 roll calls — official tallies are shown throughout. **Bill status:** congress.gov is bot-walled, so this came from GovInfo BILLSTATUS, the same Library of Congress feed, with as-of dates between November 2025 and June 2026; very recent committee action would not appear. **Not established:** no document connects the 1976 SSI 16-resident figure to the 1988 Medicaid number — that remains a hypothesis. Pre-1978 regulation (45 CFR 248.60) was never searched, because no CFR earlier than 1996 is

online; nor were the 1966 Handbook of Public Assistance Administration, the 1975–76 field instructions, or the 1982 State Medicaid Manual, each of which MACPAC records as carrying an IMD definition. So “16 had no regulatory predecessor” is proven for 1978–1991 and *unmeasured* before that. Sponsorship of the 1950 and 1972 bills is single-sourced.

Compiled 1 September 2026 · statutory text transcribed from the Statutes at Large